جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : نجوي فواد بطرس عبدالمسيح
المحافظة : الجيزه **الحي / المدينة :** العمرانيه **العنوان :** ----
رقم البطاقة : 26701092500663 **رقم التليفون :** 01122159665 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 09-01-1967 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نجوي فواد بطرس عبدالمسيح
National ID: 26701092500663

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	:الاسكندراني نوال يوسف عبدالمسيح	Request No.	: 13000356
Patient No.	:26701092500[illegible]	Request Date	: 22/05/2024
Path No.	: 1703/24	Verification Date	: 26/05/2024
Sex	: Female	Financial Category	: اورام دار السلام - فئة الاولة
Age	: 57 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 21/5/2024, 2:00 pm.

Gross Examination:
- Multiple grayish white tissue cores up to 1.2 cm long, totally submitted.

Microscopic Examination:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade II.
- Ductal carcinoma in situ: Not detected.
- Perineural invasion: Not detected.
- Angiolymphatic invasion: Not detected.

Diagnosis:
Right breast mass: Tru-cut biopsy
Invasive duct carcinoma, Grade II
For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Gamal Abd-El-Atty.
Dr. Menna Nabil

Verified by

Printing Date : 26/06/2024 09:24:09 Page 1 of 1 Printing By : menna mohamed yousef

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: نجوى فؤاد بطرس **MR. :** 420	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was diagnosed with right-sided breast cancer, clinical stage cT2N1M0, luminal A subtype. Neoadjuvant chemotherapy was administered, followed by definitive radiation therapy with wide local excision and sentinel lymph node biopsy, resulting in pathological stage ypT2N1M0. Post-operative management included placement of a port and initiation of hormonal therapy with letrozole (Femara) beginning in April 2025, subsequently combined with zoledronic acid (Zomita) and supplemental therapy. The patient continued to receive Femara plus supplemental therapy through the pharmacy on 27 March and again on 27 April.

In August 2026 the patient's hormonal therapy was paused, and in May 2026 the patient was noted to have a free supply of letrozole (Femara) for three months. The current plan is to maintain the patient on Femara for the indicated three-month period.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **Rana Essam**

يعتمد ,

Patient Name	:	Request No	: 13000366
Patient No.	: 267010[illegible]2500663	Request Date	: 22/05/2024
Path No.	: 1703/24	Verification Date	: 02/06/2024
Sex	: Female	Financial Category	: [illegible]
Age	: 57 Y	Department	: Out Patient
Referring Doc.	:	IHC No	
		Registered by	: Mrs.Mayada Tegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 21/04/2024, 2:00 pm.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
- Positive control showed appropriate reactivity.
- ER: Strong nuclear reaction in 90 % tumor cells (score 8/8).
- PR: Strong nuclear reaction in 9[illegible] tumor cells (score 8/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67: 10 % nuclear reaction of tumor cells.

Diagnosis:
Right breast mass, Tru-cut biopsy:
invasive duct carcinoma, Grade II.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10 % of tumor cells.

* Scoring system: "ER, PR"

Verified by :

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - شارع النيل - كورنيش - شارع القوطية - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mar El Qaleia - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الاسكندراني/ نجوى فؤاد يوحنا عبدالمسيح	Request No.	13000356
Patient No.	26701092500603	Request Date	22/05/2024
Path No.	1703/24	Verification Date	02/06/2024
Sex	Female	Financial Category	اورام دار السلام - نقدية الدولة
Age	57 Y	Department	Out Patient
Refereing Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoding system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within ≤ 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within ≤ 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense > 10% of tumor cells.

Signature
Prof. Dr. Gamal Abd-El-Aty
Dr. Eman Magdy.

Verified by :

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No.(4) - First Floor-Building No. 987- Nile Corniche - Masr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient's Name:	NAGWA FOUD BOTROS
Date:	2026-05-02
Referred by:	Prof.Dr.
Hospital:	Goustave Roussey International Egypt

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

INDICATION:
Right CBS

TECHNIQUE:

- Bilateral MLO and CC views
- Bilateral U/S

REPORT:

- Dense parenchyma, (ACR C), with focal disruption seen underlying the scar site with mild diffuse skin thickening.
- No suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary U/S showed no solid or cystic mass lesions.
- Normal skin thickness and nipple-areola complex.
- Left nonspecific axillary nodes are seen
- Free right axillary region as well as the infra and supra-clavicular regions.

OPINION:

- Normal study of the left breast, (BIRADS 1).
- Right post-operative changes,(BIRADS 2).

Best regards,
Dr. Yara Mohamed, MD

GUSTAVE ROUSSY
INTERNATIONAL

NAME: nagwa fouad botros
DATE:2/8 /2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (15cm at Rt MCL) bright parenchymal echo pattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with no stones or mud
- *. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

Average size, homogenous.

- **Kidneys: -**
- *Both are average in size, shape and echogenicity*
- *No stones or dilated pelvicalyceal system*
- *No gross lesion*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Not distended enough however no stones or diverticular out pouching*

Uterus and adnexal; *no abnormalities could be detected*

Opinion:

Bright liver likely fatty
Colonic gaseous distention

Best regards
Dr. NERMEEN RAMADAN FEBR, MD

Patient's Name:	NAGWA FOUD BOTROS
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Hospital:	Goustave Roussey International Egypt

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